

Teaching Philosophy:

Statement of Teaching Philosophy

My Education Mission

As a clinical pharmacist, my primary objective as an educator is to help transition pharmacy students from passive learners into independent, critically thinking professionals. My goal as an educator is to ensure that the concepts we discuss are not just memorized for an exam, but can be applied independently to real-world clinical practice. In the rapidly evolving landscape of healthcare, I strive to equip students with a structured learning framework for evidence based decision-making so they can confidently advocate for optimal patient care at the bedside.

The Clinical Classroom: Interactive Methodology in Practice

To foster independent thinking, I utilize a collaborative and engaging approach to teaching. I view case presentations and discussions through the cases as a bidirectional learning opportunity where both the learner and I can challenge and evaluate our understanding of clinical practice. To ensure comprehension and bridge the gap between theory and practice. I routinely implement targeted knowledge checks and patient case examples to reiterate core concepts. Rather than lecturing statistically, I ask open-ended questions that encourage the students to share their thought processes, allowing me to evaluate their clinical reasoning in real time and guide them towards a justifiable evidence based recommendations

Cultivating a Supportive Environment and Continuous Growth

Effective learning requires a safe psychological environment where students feel empowered to acknowledge gaps in their knowledge and view them as an opportunity to grow. I emphasize open, transparent communication and establish clear expectations from day one. I am deeply committed to providing constructive, real-time feedback that is actionable and specific. I believe education is an evolving practice. I actively seek feedback from my learners to refine my own teaching methodologies, ensuring that I continuously grow as a mentor, preceptor and clinician.

“The teacher who is indeed wise does not bid you to enter the house of his wisdom but rather leads you to the threshold of your mind” - Khalil Gibran

Didactic Teaching presentation:

Lecture Title: Venous Thromboembolism (VTE) Management In Oncology Patients: From Risk Assessment to Personalized Anticoagulation

Audience: Co-residents, Interprofessional Team, Preceptors

Format: Ground Rounds

Learning Objectives

- Understand the epidemiology and clinical significance of VTE in cancer patients
- Apply risk assessment tools, like Khorana score, to guide VTE prophylaxis decisions.

- Interpret landmark data (Hokusai, SELECT-D, CARAVAGGIO, CANVAS)
- Implement current ASCO (2023), ITAC (2022) and NCCN (2025) guideline recommendations.

Teaching Artifacts and Materials

Below are the formal instructional materials developed and utilized for this didactic presentation:

- Presentation Slides: (attached to email separately)
- Learning Strategy: Case-based patient presentations comparing DOACs vs LMWH in cancer patients with different bleeding risk

Reflection and Self Evaluation:

Presenting on oncology associated with VTE allowed me to practice translating complex and sometimes conflicting guideline information into clear, concise and easy to digest clinical information to guide actionable clinical decisions. I utilized patient cases to highlight the nuances of bleeding risks, which kept the audience highly engaged. In the future, I want to incorporate a visual decision tree handout to help learners navigate the choice of anticoagulant more efficiently.

Lecture Title: IV vancomycin dosing for Sepsis in Peritoneal Dialysis

Audience: Co-resident, Preceptors, Staff Pharmacist

Format: Formal Clinical Presentation

Learning Objectives:

- Review the diagnostic criteria and pathophysiology of sepsis.
- Explain how to classify the severity of sepsis
- Understand vancomycin dosing in peritoneal dialysis (PD)

Teaching Artifacts and Materials

Below are the formal instructional materials developed and utilized for this didactic presentation:

- Presentation slides: (attached to email separately)
- Learning Strategy: Case presentation walkthrough calculating dose adjustments based on residual renal function and peritoneal membrane characteristics

Reflection and Self-Evaluation

Renal dosing in peritoneal dialysis and sepsis is a complex topic for learners. By breaking the presentation down step by step into easy digestible concepts by providing some background information before diving into the drug dosing, I was able to make it easy to follow. The interactiveness of the case makes it easy to follow and apply these dosing strategies to an ICU patient work up.

Small Group Facilitation

During my residency training I served as an academic facilitator for the college of pharmacy, leading structured, interactive small group discussions for both foundational and advanced students.

Course 1: Basic Patient Assessment & Medication Therapy management (MTM)

Learner Level: Purdue First year Pharmacy Students (P1)

Format: Skills laboratory evaluating Interview skills

Focus and Methods:

- Foundational Skills Development: Guided introductory students through the core components of patient assessments, including taking accurate medication histories and identifying basic drug related problems
- Facilitated mock patient interviews
- Provided real-time feedback

Artifacts and Materials

- Facilitation Materials: MTM teaching docx attached in email

Course 2: RX 517: Advanced Pharmacotherapeutics

Learner Level: Butler University (Third year Pharmacy Students - P3)

Format: Advanced Clinical Case Facilitator

Focus and Methods

- Led small groups through complex, multi-disease state patient cases that required balancing guideline recommendations with patient nuances (renal impairment, comorbidities, drug to drug interaction).
- Challenged students to defend their therapeutic recommendations using guideline recommendations, drug references
- Utilized open-ended questions to prompt students to evaluate why a specific drug or dose was selected, pushing them to think like a clinical pharmacist.

Artifacts and Materials

- Facilitation Materials: Example of Case presentation on Neutropenic Fever

Reflection and Self Evaluation:

Facilitating across two completely different learner levels has expanded my versatility as an educator. With the first year students, I learned the importance of patience and building confidence, focusing heavily on foundational communication and the basics of patient care. With the advanced RX 517 students, my role shifted to a facilitator, where the key to success was stepping back and allowing the student to clinically review and collaborate with their peers. Navigating both environments has prepared me to effectively precept and mentor any level of learner I encounter in my future clinical practice.

Experiential Precepting and Co-Precepting

During my residency, I have actively precepted and co-precepted diverse levels of learners across various settings, utilizing specific coaching methods to match each student's professional needs.

Learner Level: APPE P4 Student

Setting: Oncology Service - Chemotherapy Induced Nausea and Vomiting Management

Precepting Methods:

- Active shadowing and Modeling: The P4 student shadowed me on the oncology service to observe how pharmacists evaluate patient regimens and interact with the oncology team
- During our topic discussion, the students presented core clinical concepts directly from their personalized notes. To elevate their understanding, I integrated a slide deck from a formal CINV presentation I had previously developed, walking them step by step through the treatment algorithms, dosing nuances and neuroreceptor targets.
- I provided immediate real-time verbal feedback following the discussion, highlighting the student's strong grasp of foundational drug mechanisms while guiding them on how to tailor antiemetic regimes to specific patient risk profiles.

Artifacts and Materials: CINV (attached to email sent)

Precepting Example 2:

Learner Level: P1 Student

Clinical Context: Intensive Care Unit Service

Precepting Methodologies in Action

- Introduced first year pharmacy student to foundation hospital pharmacy mechanics by demonstrating how to safely and effectively review and verify clinical orders within the electronic medical record for critically ill patients
- The P1 student accompanied me during multidisciplinary ICU rounds. I served as an active facilitator, answering their real-time questions about ICU supportive care bundles and explaining clinical rationale behind provider bedside recommendations.
- We actively reviewed clinical consults together. I walked the student through the patient profile, demonstrating how to gather data, evaluate organ function and formulate initial recommendations, giving them hands-on experience with real world clinical practice.

Artifacts and Materials: Student Feedback (attached to email sent)

Reflection and Self Evaluation

Navigating these two different teaching interactions has been interesting and formative to my growth as an educator. Presenting the introductory P1 student taught me the importance of breaking down complex fast-paced ICU workflow into basic foundational knowledge that is easy

to understand, encouraging learning moments. Conversely, working with the advanced P4 student on oncology rotation, allowed me to step into a true coaching role, utilizing my own clinical knowledge to challenge their reasoning and providing immediate actionable verbal feedback to refine their autonomy. These experiences have fully prepared me to independently precept with confidence, and elevate any level of pharmacy learner in my future clinical practice.

Preceptors/ Student Feedbacks:

Reflection on Preceptor Feedback and Professional Growth

Experiencing growth as a clinical educator requires a willingness to invite, accept and actively implement constructive feedback. Throughout my residency training, initial evaluations from my preceptors noted that while my clinical knowledge and core content delivery were strong, I occasionally struggled with presentation mechanics, specifically moving through presentation too fast and exhibiting natural shyness when commanding a room. Early on, navigating complex clinical rotations alongside teaching requirements meant balancing prep time was a significant learning curve for me.

Rather than viewing these critiques as barriers, I utilized them as a blueprint for deliberate personal and professional development. I established a consistent feedback loop with my Residency Program Director (RPD), seeking his regular feedback and mentorship on my communication style, pacing and preparation strategies. Through this continuous guidance and intentional practice, I learned to slow down my delivery, project confidence and structure my presentation efficiently.

As noted by my RPD, my trajectory throughout the year has been constantly improving. Facing these early challenges head-on has not only made me a better presenter but it has also shaped my precepting philosophy. Because I know firsthand what it feels like to struggle with confidence. I am better equipped to recognize these traits in my own students, allowing me to provide the same supportive, transformative mentorship that was extended to me.

Artifacts and Materials:

Sent you screenshots of evaluation on whatsapp